Drowning / Submersion Injury

TE-03 | Toxicology & Environmental

Quick flow

01 Safe Water Rescue [EMT / AEMT / Paramedic]

Only trained/equipped rescuers enter hazardous water. Trained responders may provide in-water breaths only when safe and without delaying removal.

02 Two Initial Breaths [EMT / AEMT / Paramedic]

For apnea or unresponsiveness, provide two initial breaths/ventilations by the first effective means available; do not delay for routine suctioning.

03 Pulse? [EMT / AEMT / Paramedic]

No: begin CPR with breaths and compressions, then use the adult or pediatric arrest pathway. Yes: continue oxygenation and airway support.

04 Trauma Indication? [EMT / AEMT / Paramedic]

Use spinal-motion restriction only when indicated and never at the expense of ventilation/CPR.

05 Dry + Warm + Monitor [EMT / AEMT / Paramedic]

Remove wet clothing; assess glucose, ECG, hypothermia, and pulmonary status.

06 Transport / Observe [EMT / AEMT / Paramedic]

Encourage evaluation, especially for cough, foam, dyspnea, abnormal sounds, or hypoxia.

Overview

- Drowning is respiratory impairment after submersion or immersion; reversal of hypoxia is the priority.
- Begin with two initial breaths/ventilations, then support ventilation and circulation without delaying for routine spinal restriction or copious airway foam.

Provider-Level Actions

Protect rescuers, stop exposure when safe, support ABCs, and involve Poison Control or Medical Control early.

EMT

- Prioritize rescue safety, give two initial breaths/ventilations when indicated, ventilate through foam without routine suctioning, begin CPR with breaths and compressions if pulseless, and dry/warm the patient.

AEMT

- Perform all EMT care plus IV/IO access and treatment of glucose abnormality, shock, or associated hypothermia.

Paramedic

- Perform all prior care plus advanced airway/cardiac monitoring and consultation for dive-related barotrauma or decompression illness.

Indications

- Any submersion/immersion event with respiratory symptoms, altered mental status, apnea, or arrest.

Warnings / precautions

- Do not waste time suctioning airway foam; ventilate through it and suction water/vomit only when needed.
- Spinal-motion restriction should never delay oxygenation, ventilation, or CPR.

Clinical pearls

- Do not use a single submersion-time cutoff alone to determine whether to begin or continue resuscitation; use the applicable cardiac-arrest and termination pathway with the full clinical context.

Source and references

Claiborne County EMS approved protocol manual. | Poison Help national line: 1-800-222-1222. | Current Tennessee EMS scope of practice and Claiborne County standing orders control for all provider permissions.

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